

Understanding and Questions

Understanding so far

- The office runs one dentist across several rooms at once, about three, with the dentist doing only the part of each treatment that needs a dentist while assistants and hygienists handle the rest.
- There are three dentists now, up from two, across about eight rooms, roughly five for cleanings and two for exams. Rooms are shared to whichever dentist is free, the team changes from day to day, and cover is arranged when someone is out.
- The day is booked in 30-minute blocks, arranged around what each dentist prefers to do, and every dentist can do every treatment.
- Rough treatment lengths are a filling and an extraction about an hour, a cleaning about 50 minutes with a 5 to 10 minute dentist check inside it, a root canal about 90 minutes to 2 hours, and an implant about 2 hours. A typical patient is about an hour, and the day runs 8 to 10 hours. The treatment names and times are our reading of the notes and should be confirmed.
- On a filling, the dentist preps and the assistant then completes it. The dentist also has to be free to check the cleanings, which ties the dentist's time to the hygiene schedule.
- One to two slots a day are kept open for emergencies.
- No-shows are filled by phone, often last minute, within a couple of days, using an ASAP list and an incentive.
- The dentist and the hygienist each have their own revenue targets, and the main goal is meeting production.

Our questions

- **Procedures, times, and breakdown.** A list of all the treatments the office does, each with its typical length, how much longer a hard case runs, and how the time inside it breaks into assistant/hygienist prep, doctor work, and assistant/hygienist finish.
- **When the doctor is needed within a procedure.** For each treatment, whether the doctor is needed at the start, middle, or end, and for how long.
- **The overlaps.** How the doctor actually moves between rooms. While the doctor is doing the doctor part in one room, what is happening in the others. How many treatments the doctor keeps going at once. For example: how the hygiene exam is slotted into a cleaning that is already running.
- **The combinations across providers.** The real mixes that run at the same time across the dentists, hygienists, and assistants. For example, a doctor on a crown while an assistant preps a filling next door and a hygienist runs a cleaning. Whether these combinations change by dentist or by day, and how the work divides between hygienists and dentists.
- **Revenue.** What each treatment is worth, as a figure, the revenue per dentist per hour, and the separate targets the doctor and the hygienist each work toward.
- **Records that may already exist.** Whether the office keeps the per-doctor procedure and time records from the last several months, and whether we can use them, since that could cover much of the timing.
- **Turnover and lab.** How long it takes to reset a room between patients, and how long the lab takes to return a crown for the second visit.
- **The mix and the team.** How often each treatment comes up in a day or week, how many assistants there are per dentist, and how many rooms one dentist runs at once.
- **Disruptions.** The no-show and cancellation rates, and how many emergencies or walk-ins on a normal day.
- **A real day to check against.** One real day or week with names removed, showing the gaps and no-shows, plus the current numbers for how full the chairs stay, patient wait, and daily production.